A/Prof Chamara Basnayake
MBBS, PhD, FRACP
Gastroenterologist
PN 421926PK

FOCUS GASTROENTEROLOGY

13-4-2026

Dr Christina Illing
Andrew Place Clinic
9 Judith Street
BUNDOORA VIC 3083
Fx:
Email:

Dear Christina,

RE: Mrs Leigh-Anne Kennedy DOB: 29/04/1977 Ph: 0407 696 320 / 0407 696 320
6 Mari Close GREENSBOROUGH VIC 3088

Summary

- ***Intermittent lower left quadrant pain and monthly bouts of diarrhoea.***
- ***Worsening gastro-oesophageal reflux and dysphagia.***
- ***Restarting pantoprazole and low-dose nortriptyline.***
- ***Repeat bloods and faecal calprotectin requested.***
- ***No action required by you.***

I had the pleasure of conducting a telehealth review with Leigh-Anne Kennedy. We primarily discussed the management of her Crohn's disease and a recent escalation in her upper gastrointestinal symptoms.

Regarding her lower gastrointestinal tract, Leigh-Anne reports generally stable function, though she continues to experience a bout of diarrhoea approximately once per month. Of note, she describes a nagging pain in the left lower quadrant that appears to intensify in the week or two leading up to her scheduled biologic injection. Her most recent C-reactive protein was reassuringly low at less than 3 mg/L. We also discussed her transition from Stelara to the biosimilar Steqyma. There have been some inconsistencies regarding the delivery of the medication through the patient support programme, and I will have my clinical nurse follow this up to ensure her supply is consistent.

Leigh-Anne's most significant current concern is an escalation of reflux and dysphagia. She describes frequent heartburn and a globus sensation. She also reports mid-chest pain upon swallowing and significant reflux that is exacerbated by exercise. She has previously been prescribed pantoprazole but has not been taking it regularly, instead opting for occasional over-the-counter esomeprazole. Furthermore, while nortriptyline was previously helpful for her bloating, she discontinued it due to xerostomia.

Impression and Plan

The current clinical picture suggests that while her inflammatory bowel disease is likely stable, her gastro-oesophageal reflux disease requires more robust management. I have encouraged her to commence pantoprazole daily. To address her chronic bloating and sleep, we will also trial a very low dose of nortriptyline, which she can titrate slowly to mitigate the risk of dry mouth. She will continue using her budesonide slurry for now, though we may cease this if her symptoms improve on the proton pump inhibitor.

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I have requested a repeat faecal calprotectin and blood tests to ensure there is no subclinical inflammation explaining her cyclical pain. While her surveillance gastroscopy and colonoscopy are currently slated for next year, I have informed Leigh-Anne that we may bring these procedures forward if her dysphagia or abdominal pain does not respond to the current medical strategy. I have provided the necessary prescriptions and will review her in four months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Patient Name: Kennedy, Leigh-Anne DOB: 29/04/1977
Recipient: Dr Christina Illing Date of Report: 14/04/2026

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065